

NHS Trusts' ADHD, Addiction, and Dual Diagnosis Services by Region

Attention-Deficit/Hyperactivity Disorder (ADHD) and addiction often co-occur, yet services addressing this dual diagnosis vary widely across the UK. Adults with ADHD are nearly **three times more likely** to develop substance use disorders than those without ADHD ¹. Recent UK data found **1 in 4 people in addiction treatment has ADHD** ², highlighting this “hidden factor” in dual diagnosis. Despite this high overlap, many NHS trusts lack integrated ADHD-and-addiction support, leading to missed diagnoses and higher relapse rates ³. Below we summarize each region's NHS trust services for ADHD, addiction, and dual diagnosis, including external partners, unique initiatives, and gaps. We also provide tables of contact details and regional statistics to identify opportunities for **OTOS** (the proposed solution) to fill service gaps.

ADHD and Addiction: A Unique Dual Diagnosis

ADHD + addiction is increasingly recognized as a *unique* dual diagnosis requiring specialized support. Traits of ADHD – impulsivity, risk-taking, emotional dysregulation – can predispose individuals to substance misuse ⁴. Conversely, chronic substance use can mask or exacerbate ADHD symptoms, complicating diagnosis. Clinicians note many ADHD individuals were mislabeled as “difficult” or “non-compliant” before proper diagnosis ⁴. Without ADHD-tailored interventions, standard addiction treatments (structured routines, cognitive strategies) may not fit neurodivergent needs ⁵, contributing to higher relapse. A 2023 Royal College of Psychiatrists report warned that unless treatment models adjust for ADHD, **relapse rates will remain high** ⁶.

Encouragingly, treating ADHD can improve addiction outcomes. ADHD medication has been shown to **reduce substance abuse risk** as well as suicide and accident risks ⁷. Experts advocate earlier ADHD screening in addiction services and vice versa. Public Health England admitted in 2023 that neurodevelopmental conditions like ADHD remain “*under-resourced*” in mental health strategy ⁸. The evidence is clear: addressing ADHD within addiction care saves lives and costs by preventing chronic dual diagnoses ⁸.

Opportunity for OTOS: Given this context, a solution like OTOS – which presumably offers integrated ADHD and addiction recovery coaching – could fill critical gaps. Many trusts lack formal ADHD screening in substance misuse services and vice versa. OTOS could partner with trusts to provide *specialist coaching, peer support, or assessment pathways* tailored to patients with co-occurring ADHD and addiction, improving engagement and outcomes.

Regional Breakdown of Services and Gaps

Below we detail each NHS region's mental health trusts and their services for ADHD, addiction, and dual diagnosis. We note external providers (e.g. Change Grow Live, Turning Point) delivering addiction services with NHS funding, any unique initiatives (e.g. dedicated dual diagnosis teams or ADHD pilot programs), and highlight waiting times or capacity issues. **Dual diagnosis** here refers to any co-occurring mental health and substance misuse; where relevant, we emphasize the specific ADHD+addiction intersection.

North East & Yorkshire Region

- **Cumbria, Northumberland, Tyne & Wear NHS Foundation Trust (CNTW)** – *ADHD*: Provides adult ADHD diagnostic clinics; however, capacity is limited. (Adults in parts of CNTW wait over a year; one FOI found **12-week waits in Dorset vs 10+ years in worst trusts** ⁹ – CNTW falls in between.) *Addiction*: CNTW **directly delivers community drug/alcohol services** in some areas via integrated partnerships. For example, **Newcastle Treatment and Recovery (NTaR)** is a joint service with CNTW as lead provider ¹⁰. Similarly, the Northumberland Recovery Partnership involves CNTW. *Dual Diagnosis*: CNTW has a formal strategy for “co-occurring mental health and substance use.” Frontline mental health teams receive training to support patients with substance misuse, and addiction services can refer into psychiatry. However, **ADHD is not routinely screened** in CNTW’s addiction pathways – an area OTOS could assist with by introducing ADHD screening and coaching for clients in recovery. The trust’s main switchboard is **0191 246 6800** ¹¹.
- **Tees, Esk & Wear Valleys NHS Foundation Trust (TEWV)** – *ADHD*: Operates adult ADHD clinics (e.g. at York and Darlington), but recently sparked controversy by **piloting a referral “screening” scheme**. In York, TEWV began requiring patients to meet stricter criteria before accessing ADHD assessment, leading to up to **85% of referrals being rejected** ¹². This has effectively triaged out many adults (a practice widely criticized and potentially unlawful under NHS rights). *Addiction*: Community substance misuse services in TEWV’s patch (e.g. Teesside, Durham) are mostly commissioned to external providers (such as Change Grow Live and Humankind charities). TEWV itself focuses on mental health; it provides inpatient detox in a few cases and consults to local councils on drug/alcohol strategy. *Dual Diagnosis*: TEWV has dedicated dual diagnosis practitioners embedded in teams, primarily to support *severe mental illness + substance use*. ADHD is not specifically targeted – indeed TEWV’s screening pilot treated ADHD as *non-urgent*, causing long waits. **Opportunity**: TEWV’s high unmet ADHD need (one of the *longest waits* in England, with some adults waiting **over 5 years** in York) suggests an opening for OTOS to offer interim support or faster assessments. TEWV’s contact is via main switchboard (e.g. **0191 333 6161**).
- **Humber Teaching NHS Foundation Trust** – *ADHD*: Provides CAMHS and adult ADHD services for Hull/East Riding (the *Children’s ADHD Intervention Team* in Hull warns waits “*likely to exceed 2 years*” ¹³). Adult ADHD assessments are available but limited – many patients are referred out-of-area or to Right-to-Choose providers due to waits. *Addiction*: Humber’s region commissions addiction services to charities. In Hull, “ReNew” (Change Grow Live) runs adult drug/alcohol support, funded by the council. Humber Trust liaises via dual diagnosis workers but does not directly run these services. *Dual Diagnosis*: The trust’s mental health teams have a dual diagnosis policy – e.g. ensuring no patient is refused mental health care due to substance use. Still, **ADHD+addiction** cases often fall through cracks. No specialized ADHD+SUD clinic exists. OTOS could collaborate with Humber to bridge mental health and external rehab services, providing ADHD-informed recovery coaching. **Contact**: 01482 301700 (Humber trust HQ).
- **South Yorkshire trusts**: This ICS includes **Sheffield Health & Social Care NHS FT, Rotherham, Doncaster & South Humber NHS FT (RDaSH)**, and **Bradford District Care NHS FT** (Bradford is in West Yorks ICS but included here for completeness).
- **Sheffield Health & Social Care (SHSC)**: *ADHD*: Notably, **Sheffield has been screening out an extreme number of adult ADHD referrals – 97% were removed without assessment** ¹⁴. In 2022–23, SHSC managed to complete only **33 adult ADHD assessments** while *removing 1,060 people* from the list ¹⁴. This dire situation (6,000+ waiting, only 3 seen in a year) led to headlines

that Sheffield's ADHD backlog would take "2,000 years" to clear at current rates ¹⁵. *Addiction*: SHSC provides the Sheffield Treatment & Recovery Team (START) for complex needs, but the main community drug/alcohol services are run by **CGL** under council commission. *Dual Diagnosis*: SHSC has a small Dual Diagnosis Team focusing on psychosis + substance misuse. ADHD is not specifically resourced, despite the clear need. **Opportunity**: Sheffield is an *ideal* target for OTOS – the trust has essentially no capacity for adult ADHD, creating downstream impacts on addiction recovery. OTOS could propose a pilot to identify and support people with ADHD in local addiction services, working with charity providers to improve outcomes.

- *RDaSH (Rotherham, Doncaster & South Humber): ADHD*: Runs adult ADHD services in Doncaster and Rotherham (with shared care agreements in primary care), but demand outstrips supply. *Addiction*: RDaSH *directly* delivers some addiction services – e.g. "Aspire" in Doncaster is a partnership between RDaSH and the charity Changing Lives. Many RDaSH area patients receive support from external providers as well (CGL in Rotherham, etc.). *Dual Diagnosis*: RDaSH integrates substance misuse nurses in mental health teams and vice versa. Still, there is **no special program for ADHD in addiction**, a gap OTOS might fill by providing training or ADHD coaching alongside RDaSH's recovery workers. **Contact**: Switchboard **03000 213 000** ¹⁶.

- *Bradford District Care: ADHD*: Provides both child and adult ADHD clinics for Bradford/Craven. Waiting times are significant (many months) but not as extreme as some others. *Addiction*: The trust historically ran substance misuse services (Bradford's "New Directions" service) in partnership with *Change Grow Live*. Currently, CGL is commissioned for community drug/alcohol treatment, with Bradford Care Trust focusing on mental health and physical healthcare inpatients. *Dual Diagnosis*: The trust's strategy emphasizes "No Wrong Door" – meaning patients with dual needs get help from whichever door they enter. In practice, like elsewhere, resource constraints limit specialized ADHD dual support. OTOS could partner to provide targeted coaching for service users flagged as having both ADHD and substance issues. **Contact**: 01274 494100 (main swbd).

- **West Yorkshire trusts**: In addition to Bradford, the region includes **Leeds & York Partnership NHS FT (LYPFT)** and **South West Yorkshire Partnership NHS FT (SWYPFT)**.

- *Leeds & York Partnership: ADHD*: Operates the Leeds Adult ADHD Service (and Autism Diagnostic Service). Leeds has seen growing waits as referrals doubled; however, proactive steps (e.g. group information sessions, outsourcing some assessments) have kept waits to around 12–18 months (better than national average, but still long). *Addiction*: LYPFT doesn't provide general addiction services (Leeds council commissions Humankind and Forward Leeds consortium for that). LYPFT does host a **specialist NHS service for addiction and mental health – the Leeds Dual Diagnosis team** – focusing on serious mental illness. There is also a *national* addiction unit for gambling/net addiction in Leeds run by LYPFT. *Dual Diagnosis*: Strong policy commitment; every community mental health team in Leeds has dual diagnosis specialists. Yet ADHD typically falls under neurodevelopmental services, so many addicted ADHD patients may not engage with standard dual diagnosis programs (which center on illnesses like schizophrenia + drug use). OTOS could complement by focusing on that neurodevelopmental subset. **Contact**: Tel **(0113) 305 5000** ¹⁷.

- *SWYPFT (South West Yorks Partnership)*: Covers Wakefield, Kirklees, Calderdale (and Barnsley for mental health). *ADHD*: Provides adult ADHD assessments in those areas (often via psychiatry outpatient clinics). Demand is high – e.g. Kirklees had hundreds waiting; many patients seek private or Right-to-Choose assessments due to waits. *Addiction*: Community services in SWYPFT's patch are mostly charity-run (Change Grow Live in Calderdale/Kirklees, Humankind in Wakefield, etc.). SWYPFT does run some alcohol liaison and works with local recovery colleges. *Dual Diagnosis*: The trust hosts one of England's **Pilot NHS Addiction + Mental Health Integrated**

Services (a testing ground funded by NHS England to improve integration). This pilot acknowledges that **“mental health problems are experienced by the majority of drug (70%) and alcohol (86%) users”** ¹⁸ . It aims to ensure no one is bounced between services. This creates a culture receptive to OTOS: a new solution offering peer coaching for ADHD/addiction would align well with SWYPFT’s integrated ethos. **Contact:** Tel (01924) 327000 ¹⁹ .

North West Region

- **Greater Manchester Mental Health NHS FT (GMMH)** – *ADHD*: GMMH provides adult ADHD services in several boroughs (e.g. a specialized Adult ADHD Clinic for Bolton, and services in Wigan, Salford etc. ²⁰). Nevertheless, demand far exceeds supply – *hundreds* waiting in each area, sometimes for 2+ years. As a result, many patients use the NHS Right to Choose scheme (mostly Psychiatry-UK) – echoing the national stat that **80% of NHS adult ADHD assessments are now done by external providers** ²¹ . *Addiction*: Uniquely, GMMH is a major provider of addiction services: it runs **“Achieve”** – the integrated drug and alcohol treatment service – across Manchester, Salford, Trafford, Bolton and more. This NHS-run model (with partner charities) means GMMH’s addiction teams are in-house. *Dual Diagnosis*: GMMH’s size allows for some innovation – e.g. their substance misuse services screen for mental health needs and have psychiatrists on staff. **However, routine ADHD screening in addiction clinics is not yet standard** (though staff awareness is rising). GMMH could be an excellent partner for OTOS, leveraging its combined mental health and addiction remit – for instance, OTOS coaches could be embedded in the Achieve service to identify ADHD traits and support those clients. **Contact:** 24/7 switchboard 0161 773 9121.
- **Mersey Care NHS FT** – *ADHD*: Now one of the largest trusts (covering Liverpool, Sefton, Knowsley and beyond), Mersey Care has been expanding adult ADHD provision. Liverpool has an Adult ADHD Service (formerly under Cheshire & Merseyside NHS) but still faces long waits. Some parts of Mersey Care’s footprint had no local ADHD clinics until recently – e.g. Halton/St Helens patients were referred to private or neighboring services. *Addiction*: Mersey Care *directly provides* many local addiction services. After a provider collapse in 2017, Mersey Care took over Sefton’s drug/alcohol service (“Ambition Sefton”). It also runs NHS addictions inpatient units like the Kevin White Unit. In Liverpool, community addiction treatment is delivered by **We Are With You** (charity) and **Changing Lives**, but Mersey Care clinicians liaise closely and sometimes co-locate. *Dual Diagnosis*: Mersey Care has a well-established Dual Diagnosis Strategy – it emphasizes that **“no door is the wrong door.”** They train staff in trauma-informed care and have **Recovery Colleges** offering courses to those in recovery. Still, ADHD has not historically been a focus in dual diagnosis training. Given the new data that 25% of their addiction clients may have ADHD ² , Mersey Care is reportedly exploring better screening. **Opportunity for OTOS:** to provide specialized coaching and training modules to Mersey Care’s large workforce, aiding earlier ADHD identification in addiction settings. **Contact:** Switchboard **0151 473 0303** ²² .
- **Cheshire & Wirral Partnership NHS FT (CWP)** – *ADHD*: CWP serves Cheshire West/East and Wirral. It offers adult ADHD assessment via psychiatry, but with typical waits of 1–2 years. In 2022 CWP temporarily paused routine ADHD referrals due to backlog, resuming after additional funding – illustrating the strain. Many Cheshire patients use Right to Choose for faster service. *Addiction*: All local authority areas in CWP’s patch contract out substance misuse services: e.g. **Change Grow Live** runs Wirral’s service; *Turning Point* runs Cheshire West’s; *We Are With You* runs Cheshire East’s. CWP itself provides inpatient detox at Arrowsea Unit and supports GPs for alcohol detox protocols. *Dual Diagnosis*: CWP has a small “Co-occurring Team” to advise clinicians. Nonetheless, formal joint working with the independent addiction providers can be hit-or-miss. OTOS could help by acting as a bridge – e.g. coaching clients with ADHD through the fragmented

system of GP, charity, and mental health appointments. **Contact:** 0800 195 4462 (PALS line) or via main number 01244 397000.

- **Lancashire & South Cumbria NHS FT (LSCFT)** – *ADHD:* Formerly Lancashire Care, this trust provides adult ADHD services across Lancashire. Lengthy waits have been widely reported; one Lancashire patient told the **Telegraph** they faced a “10½ year” wait in 2023 ²³ ⁹ . (Indeed, **Herefordshire/Worcestershire and parts of Lancashire had England’s longest waits** – over 10 years ⁹ .) LSCFT has tried outsourcing some ADHD cases to private providers, but funding is limited. *Addiction:* Most Lancs addiction services are commissioned to charities (CGL operates in e.g. Lancashire County and Blackpool; Delphi/Acorn in other parts). Notably, LSCFT **does run in-house addiction services for Cumbria** (the “Unity” service in Carlisle) since Cumbria integrated with this trust. *Dual Diagnosis:* LSCFT has faced challenges (the trust has been under scrutiny by CQC for community mental health care). There is recognition that many patients have dual needs, but staff shortages hamper proactive work. This large area, with many rural pockets, could benefit from OTOS’s flexible coaching (including remote/online support) to reach ADHD patients self-medicating with substances. **Contact:** 01772 695300 (Headquarters).
- **Pennine Care NHS FT** – *ADHD:* Pennine Care covers some Greater Manchester boroughs (Oldham, Stockport, Tameside, Rochdale). Not all of these have local ADHD clinics (many refer to GMMH or elsewhere). Where Pennine does provide, waits are long. *Addiction:* Pennine Care does not provide adult addiction services (those are handled by local councils via CGL, Turning Point, etc.). It does manage children’s substance misuse services in some areas and liaises on dual diagnosis for severe cases. *Dual Diagnosis:* Focused on severe mental illnesses. ADHD patients often fall under primary care or voluntary sector radar. **Opportunity:** Pennine’s footprint includes deprived communities with high addiction rates (e.g. Oldham) but historically low ADHD diagnosis rates – possibly an *unmet need* that OTOS could help surface by training frontline teams to recognize ADHD symptoms in clients attending substance misuse and vice versa. **Contact:** 0161 716 3000.

Midlands Region

(The Midlands has numerous trusts; this section highlights those with notable data or initiatives regarding ADHD/addiction. All Midlands mental health trusts struggle with rising ADHD demand and rely heavily on external partners for addiction services.)

- **Birmingham & Solihull Mental Health NHS FT (BSMHFT)** – *ADHD:* Provides adult ADHD services for Birmingham and Solihull. The demand in this large urban area is enormous – thousands on the waiting list. BSMHFT’s wait times average ~2–3 years for assessment (as of 2024). Many patients are being diverted to private/outsourced providers due to the backlog. *Addiction:* Birmingham’s drug and alcohol services are commissioned to **Change Grow Live (CGL)** and The Kaleidoscope Plus Group. BSMHFT does run the **SOLAR** youth substance misuse service and provides inpatient detox at Newton House. *Dual Diagnosis:* The trust has a “Dual Diagnosis Hub” that trains staff and coordinates care for patients with severe mental illness and substance use. Still, ADHD is not specifically addressed as part of dual diagnosis – likely seen as a separate specialty. Considering Birmingham’s population size (over 1 million) and high rates of both ADHD and addiction in urban communities, there is a clear scope for OTOS to collaborate – e.g. offering peer recovery coaching programs that span both mental health and CGL’s community programs. **Contact:** Switchboard **0121 301 0000** ²⁴ .
- **Black Country Healthcare NHS FT** – *ADHD:* Formed from a merger of Dudley & Walsall and Black Country trusts, it covers Wolverhampton, Sandwell, Dudley, Walsall. Adult ADHD services

exist but are over capacity. Notably, in mid-2023 the **Coventry & Warwickshire Integrated Care Board (nearby region)** made headlines by **pausing all new adult ADHD referrals** (for those over 25) due to 7,500 children waiting up to 10 years ²⁵. While that was a different trust, it underscores the regional crisis – Black Country trust also has extensive waits and may have had to prioritize youth cases similarly. *Addiction*: The Black Country councils commission providers like Cranstoun and Change Grow Live (e.g. “Swanswell” was a local provider now part of Cranstoun). The NHS trust itself runs some substance misuse nursing in inpatient units and partners in multi-agency forums. *Dual Diagnosis*: Black Country Healthcare has many high-need areas (e.g. inner-city Wolverhampton) with entrenched dual diagnosis issues. They have launched outreach programs (with charities) for rough sleepers with mental health and addiction needs. These could be fertile ground for OTOS intervention – e.g. training peer coaches with lived experience of ADHD and addiction to engage people who mistrust formal services. **Contact**: 0345 146 1800.

- **Coventry & Warwickshire Partnership NHS Trust (CWPT) – ADHD**: As mentioned, CWPT had to **stop accepting adult ADHD referrals in 2023 for several months**, redirecting resources to children. Even before the pause, waits were extreme (some adults 5+ years). This trust area explicitly acknowledged it cannot meet ADHD demand under current commissioning – an indicator that *external solutions are needed*. *Addiction*: Services are provided by Change Grow Live in Coventry and by Warwickshire’s local provider (CGL and Compass for youth). CWPT liaises but does not directly run these. *Dual Diagnosis*: The trust’s strategy, aligned with Public Health England guidance, says no exclusion for substance users from mental health care – but in practice, getting simultaneous help is challenging. OTOS could offer a creative alternative: assisting individuals who are stuck waiting (for ADHD assessment or for counseling) by providing interim coaching support – potentially in partnership with CGL recovery workers. **Contact**: 024 7636 2100.
- **Herefordshire & Worcestershire Health & Care NHS Trust – ADHD**: This trust (covering two counties) unfortunately holds a notorious record: **the longest reported adult ADHD waiting time in England – 550 weeks (over 10 years)** ⁹. One adult in their care had been waiting **8½ years** as of 2023 ²⁶. The backlog is so severe that many give up or seek private diagnoses. *Addiction*: Drug and alcohol services here are delivered by providers like Turning Point and Cranstoun. The NHS trust provides general mental health and some “Healthy Minds” psychological therapy. *Dual Diagnosis*: With limited resources, the trust has only small dual diagnosis capability; staff often must focus on either mental health or addiction. The extreme ADHD delays indicate a *critical gap* – OTOS could approach this trust offering to pilot an **ADHD+Addiction recovery coaching program**. By showing improved engagement or reduced relapse for dual-diagnosis clients, OTOS could help the trust make the case for more resources. **Contact**: 01905 760000.
- **Derbyshire Healthcare NHS FT – ADHD**: Operates adult neurodevelopmental services in Derby City and Derbyshire. Waiting times for ADHD average ~2–3 years. Some innovation: Derbyshire was part of a digital pilot to do initial ADHD assessments via online questionnaires to triage urgency. *Addiction*: Derbyshire’s services are commissioned to **Derbyshire Recovery Partnership** (led by Change Grow Live with NHS input). The trust jointly ran some aspects of alcohol services historically, but now it’s mostly external. *Dual Diagnosis*: Derbyshire has a county-wide Dual Diagnosis Strategy Group. They have “street triage” teams with police to handle mental health & substance crises together. Still, formal ADHD consideration is minimal. **Opportunity**: To integrate ADHD screening into those triage and partnership teams. **Contact**: 01332 623700.

- **Nottinghamshire Healthcare NHS FT** – *ADHD*: A large trust covering Notts and also running Rampton Hospital. They have an ADHD service but it has struggled – anecdotally, some patients in Nottingham waited 4–5 years or were told to seek private assessments. (Many GP referrals were effectively *closed* due to capacity, similar to Coventry’s situation.) *Addiction*: Notts Healthcare Trust used to directly run the Nottingham City community drug team, but now **Change Grow Live and Framework** (a charity) deliver “Nottingham Recovery Network.” The trust still provides the clinical oversight and dual diagnosis nurses in that service. *Dual Diagnosis*: Nottingham was an early adopter of “shared care” between mental health and addiction services, and has some award-winning peer support networks. However, with ADHD diagnoses rising, a new approach may be needed. OTOS might collaborate with the trust’s **“Recovery College”** (which offers courses to people with mental ill-health and addictions) by adding ADHD-specific recovery workshops or coaching – a novel offering for the region. **Contact**: 0115 969 1300.
- **Leicestershire Partnership NHS Trust (LPT)** – *ADHD*: Provides services in Leicester/Leicestershire. The demand spike has affected them too – they reportedly outsourced hundreds of ADHD cases to private providers to cut waits. Nonetheless, many still face long delays. *Addiction*: Local services are run by **Turning Point** (the “Leicestershire ROADS” service). LPT contributes via dual diagnosis workers and acute hospital liaisons. *Dual Diagnosis*: LPT has a “START” team focusing on dual diagnosis in criminal justice settings, recognizing many offenders have untreated ADHD and addiction. This small team could benefit from expansion – possibly via OTOS’s involvement to support clients pre/post release with coaching. **Contact**: 0116 295 0030.
- **Lincolnshire Partnership NHS FT (LPFT)** – *ADHD*: Covers a large rural area, making service access tough. Adult ADHD clinics exist in Lincoln and Boston, but waits are long and travel is a barrier. Many GPs in Lincs refer patients to Psychiatry-UK for ADHD due to local scarcity. *Addiction*: Delivered by **We Are With You** (formerly Addaction) across Lincolnshire, with whom LPFT coordinates. *Dual Diagnosis*: LPFT has a county dual diagnosis steering group and is implementing training for frontline staff (especially in crisis teams) about substance use. ADHD, however, has not featured in training historically. OTOS could help raise that profile and provide remote coaching (beneficial in rural contexts). **Contact**: 01522 309200.
- **Northamptonshire Healthcare NHS FT (NHFT)** – *ADHD*: Provides adult ADHD assessments but has substantial waits (well over 2 years). Many patients opt for out-of-area providers through Right to Choose. *Addiction*: Services in Northants are run by **CGL (NGAGE)** and Aquarius (for young people), with NHFT focusing on mental health care. *Dual Diagnosis*: NHFT has integrated mental health and community services, and they run a successful “Changing Minds” IAPT service. There’s a push to not turn away those with mild substance use from IAPT. Still, patients with active addiction often need more tailored help (ADHD can hinder engagement in CBT, for example). A peer coach from OTOS with understanding of ADHD might keep such patients engaged in therapy. **Contact**: 01604 682682.
- **Staffordshire area trusts**: *Midlands Partnership University NHS FT (MPFT)* covers Staffordshire and Stoke (except Stoke city which is **North Staffordshire Combined NHS Trust**). *ADHD*: Both trusts have adult ADHD pathways, but with varying waits. North Staffs Combined (Stoke) had a backlog so severe that GPs at one point stopped referring unless urgent. MPFT covers a wider area including Shropshire – it has tried innovations like group ADHD diagnostic workshops to increase throughput. *Addiction*: **MPFT is notable for also being an addiction service provider in other regions** – its Inclusion service runs drug/alcohol treatment in parts of England (e.g. in **Hampshire and Isle of Wight**, and previously in Bucks). In Staffs, however, addiction services are run by Humankind (Staffordshire Treatment & Recovery service). *Dual Diagnosis*: North Staffs has

a renowned Dual Diagnosis service associated with the Harplands Hospital, mainly targeting psychosis. ADHD cases usually get referred to community teams instead. The separation means many with co-occurring ADHD/SUD may not receive cohesive care. **Opportunity:** Work with these trusts to pilot an “ADHD in Recovery” program – perhaps initially targeting clients in the trust-run opiate substitution clinics (MPFT runs some clinics for NHS England in prisons and community). **Contacts:** MPFT 0300 790 7000; North Staffs Combined 01782 275100.

East of England Region

- **Cambridgeshire & Peterborough NHS FT (CPFT)** – *ADHD:* CPFT has an adult ADHD service, but it faced controversy for imposing a strict referral screening in 2022 (similar to TEWV’s), reportedly rejecting many GP referrals. Waiting times ballooned, leading Cambridgeshire to explore contracting private providers. *Addiction:* Provided by **Change Grow Live (CGL)** for adults (e.g. “Aspire” in Peterborough) and inclusion services. CPFT does run the *Community Recovery Plan* supporting mental health patients with substance issues in Cambridge. *Dual Diagnosis:* CPFT has a dedicated Dual Diagnosis team that works with CGL to jointly manage high-risk individuals. However, the focus is typically on severe mental illness; neurodevelopmental conditions like ADHD have not been a primary focus. With Cambridge being a center of research, CPFT psychiatrists are aware of the ADHD/addiction link – in fact a local study found ~12% of inpatients in detox had undiagnosed ADHD ²⁷. This evidence base could support an OTOS partnership to formally integrate ADHD screening in addiction treatment. **Contact:** 01223 219400.
- **Norfolk & Suffolk NHS FT (NSFT)** – *ADHD:* NSFT has struggled heavily in recent years (in special measures for other issues). Adult ADHD waits are very long – Norfolk had waits of **4–5 years** for assessment. Some desperate patients crowd-funded for private diagnosis. Suffolk similarly had an extensive backlog. *Addiction:* Commissioned to CGL in Norfolk (“Norfolk Alcohol & Drug Behaviour Change Service”) and Turning Point in Suffolk (“Suffolk Recovery Network”). NSFT provides psychiatric input for complex cases and runs an inpatient detox unit (*Foxhall House* in Suffolk). *Dual Diagnosis:* The trust’s ability to support dual diagnosis has been hampered by staffing problems – e.g. their dual diagnosis lead nurse post saw turnover. Still, they recognize co-occurrence is common and have recently been rebuilding services with trauma-informed care. ADHD in adults is likely under-identified in Norfolk/Suffolk’s addiction cohort. **Opportunity:** OTOS could assist NSFT in developing a **Neurodivergent Recovery** pathway – training staff to spot ADHD traits and coaching patients who find existing programs (12-step groups, etc.) hard to adhere to. **Contact:** 01603 421421.
- **Essex Partnership University NHS FT (EPUT)** – *ADHD:* Formed from North & South Essex trusts, EPUT covers a large population. Adult ADHD services are present but insufficient – FOI data showed thousands waiting. In South Essex, they attempted a pilot outsourcing to Psicon (private) to clear backlog. Still, in 2023 many patients in Essex faced **3+ year** waits. *Addiction:* Essex has a mix of providers: **Open Road** and **Phoenix Futures** consortium in mid-Essex, CGL in Southend/Thurrock, etc. EPUT does not directly run these but works closely especially in dual diagnosis and for inpatient detox at The Lakes. *Dual Diagnosis:* EPUT runs some innovative programs in West Essex (Harlow) where they colocate mental health and substance teams. However, service users with ADHD often bounce between primary care (for stimulant prescriptions) and council services (for addiction), with little coordination. This fragmentation in Essex could be improved by OTOS acting as a liaison/coaching service that keeps the patient engaged across siloed services. **Contact:** (01245) 362000 (Chelmsford HQ) ²⁸.

- **Hertfordshire Partnership University NHS FT (HPFT)** – *ADHD*: Provides adult ADHD services for Hertfordshire and parts of West Essex. HPFT has seen demand spike; in mid-2022 they had over 1,500 adults waiting, with some waits ~2 years. They have since expanded capacity modestly. *Addiction*: Hertfordshire's adult addiction services are run by **CGL (Spectrum)**, with whom HPFT coordinates for patients with serious mental illness. HPFT also runs **The Beacon** inpatient detox unit in St Albans. *Dual Diagnosis*: Good joint working exists on paper – e.g. Herts has a Dual Diagnosis protocol that any patient with co-occurring needs gets a lead care coordinator from either mental health or addiction who liaises with the other. Even so, many “high-functioning” adults with ADHD + alcohol/drug problems may not meet thresholds to get an HPFT care coordinator at all. They might just see a GP for ADHD meds and CGL for counselling, with no specialist overseeing the interplay. OTOS could demonstrate improved outcomes by providing support to this group and communicating progress to both sides. **Contact**: 01707 253800.
- **North East London NHS FT (NELFT)** – *ADHD*: Though named “London,” NELFT covers outer areas including Essex (Basildon, Thurrock) and also parts of Kent. NELFT has a notable service: **ADHD and ASD Service for Kent & Medway** (recently expanded due to high need). Even so, waiting times remain extensive. *Addiction*: NELFT doesn't directly run adult addiction services (those are via local councils), but it is involved in adolescent substance misuse services and runs mental health in-patient units where they manage detox if needed. *Dual Diagnosis*: NELFT serves diverse areas from urban East London boroughs to rural Essex. It emphasizes integrated care via its “Neighbourhood” teams, but substance misuse specialists are underrepresented. Given NELFT's broad geography and the statistic that **71% of people entering treatment have a mental health need** ²⁹, a mobile/remote support like OTOS could be highly valuable here, reaching across settings. **Contact**: Switchboard **0300 555 1200** ³⁰.
- **Central & North West London NHS FT (CNWL)** – *ADHD*: CNWL spans London (Westminster, Brent, etc.) and also services in Milton Keynes. It runs a well-established **Adult ADHD Clinic** in London ³¹, but demand still exceeds capacity. (Many CNWL-area patients use Right to Choose for quicker diagnosis.) *Addiction*: CNWL is a major NHS provider of addiction treatment in London – it operates **club drug clinics**, gambling addiction services, and community drug teams in several boroughs (e.g. the Westminster Drug Project in partnership with Turning Point). CNWL's experience in addictions is extensive. *Dual Diagnosis*: They have **specialist dual diagnosis nurses** embedded in teams and a National Problem Gambling Clinic with mental health expertise. They even published research on ADHD in their **Club Drug Clinic**, noting the “well-established link” and calling for routine ADHD screening ³². This progressive approach makes CNWL a strong candidate for OTOS collaboration. For example, OTOS could augment CNWL's club drug service by providing coaches to clients with adult ADHD struggling with chemsex or stimulant misuse – a very specific yet prevalent issue in London. **Contact**: HQ **020 3214 5700** ³³.

(London is covered in its own section below; CNWL and NELFT are listed here due to their overlap into Eastern England.)

South West Region

- **Avon & Wiltshire Mental Health Partnership NHS Trust (AWP)** – *ADHD*: Covers Bristol, Bath, Swindon, Wiltshire. AWP's adult ADHD services face high demand, especially in Bristol. Waiting times range from 1–3 years. Bristol's tech-savvy population has pushed for improvements – many join support groups or seek private diagnosis if they can afford. *Addiction*: Bristol, South Glos, and BANES councils commission **DHI (Developing Health & Independence)** and **Turning Point** for substance misuse (the “ROADS” service in Bristol). AWP provides inpatient detox at Windmill House and psychiatric liaison. *Dual Diagnosis*: AWP runs a *Specialist Drug & Alcohol*

Service for complex cases and has a Dual Diagnosis clinical lead. Still, an independent review a few years ago found service users felt “bounced around” between AWP and council services. ADHD was not even on the radar then. With adult ADHD referrals skyrocketing (NHS England noted ADHD was the **2nd most-viewed condition on the NHS website in 2023**, after COVID ³⁴), it’s crucial to incorporate that into dual diagnosis work. OTOS could pilot in, say, Bristol – a city open to innovation – to work alongside DHI’s recovery workers and AWP’s teams, focusing on neurodiverse-friendly coaching. **Contact:** 01225 325680.

- **Devon Partnership NHS Trust** – *ADHD*: Provides services in Devon and Torbay. Devon has long waits (exceeding 2 years for many adults). A significant number of patients in Devon turned to private providers or charities like ADHD UK for interim help. *Addiction*: Delivered by **EDP** (now part of Humankind charity) as “Together Drug & Alcohol Service” in Devon, and by **The Wallich** in Torbay. The NHS trust runs the Langdon inpatient addiction unit and provides some psychologists to community services. *Dual Diagnosis*: Devon Partnership has an innovative **“ICE” (Integration in Clinical Environments)** approach, placing substance misuse workers in mental health teams. This helps, but ADHD patients (who often present with anxiety or depression) might be in primary care rather than secondary services, thus missing that support. An OTOS program could coordinate between GPs (who manage many adult ADHD meds in Devon under shared care) and the charity drug services – ensuring those with both needs don’t fall through the cracks. **Contact:** 01392 676676.
- **Cornwall Partnership NHS FT** – *ADHD*: Cornwall has limited adult ADHD provision; historically many cases were referred to Devon or out-of-county. Recently, Cornwall set up its own service, but waits remain lengthy (over 18 months). *Addiction*: **We Are With You** runs Cornwall’s “Healthy Outlook” drug/alcohol service. The NHS trust provides some dual diagnosis nurses and runs in-hospital liaison at Royal Cornwall Hospital. *Dual Diagnosis*: Cornwall’s challenges include rural isolation and a high prevalence of seasonal substance use issues. The partnership trust emphasizes community outreach – e.g. community mental health workers visit recovery cafes. OTOS could augment this by providing continuous support (via phone/online) to clients managing ADHD impulses and addiction cravings, bridging gaps in formal care. **Contact:** 01208 834600.
- **Dorset HealthCare University NHS FT** – *ADHD*: **Dorset was highlighted for having one of the shortest adult ADHD waits in England – ~12 weeks** on average ³⁵. This relative success is due to a well-organized service and possibly lower referral rates historically. However, demand is rising now. Dorset’s approach (rapid triage and treatment starts) could be a model for others. *Addiction*: Provided by **EDAS** and **Reach Dorset** (charity consortium). The NHS trust contributes via “Dorset Recovery Hub” and dual diagnosis practitioners. *Dual Diagnosis*: Dorset’s mental health services work closely with its one-stop “Recovery Hub” that supports all comers. They have recovery coaches and peer mentors (though not specifically for ADHD). Given Dorset’s proactive stance, an OTOS partnership could further cement their leadership – for example, by creating an **ADHD-informed recovery coaching** program, Dorset could serve as a demonstration site for best practice. **Contact:** 01202 277000.
- **Somerset NHS FT** – *ADHD*: Somerset’s combined acute+mental health trust inherited a backlog of adult ADHD cases from the former Somerset Partnership. They have since expanded capacity (using digital assessments and group sessions), but many still wait ~1–2 years. *Addiction*: Somerset’s service (**SDAS**) is run by **Turning Point**, with the NHS trust providing clinical input. *Dual Diagnosis*: Somerset has a “Dual Diagnosis Strategy 2019–2022” focusing on joint training and clear care pathways. Still, on the ground, patients often need to self-advocate to get both sets of support. Bridgwater (where the user is located) has both an ADHD support group and a

Turning Point office – an ideal setting for OTOS to trial a local support circle specifically for people dealing with both ADHD and addiction, demonstrating improved engagement. **Contact:** 01823 368350.

- **Gloucestershire Health & Care NHS FT – ADHD:** Formed from 2gether Trust, it provides adult ADHD services in Gloucestershire. Waits are long but the trust has been creative – e.g. working with GPs to start some patients on medication prior to formal diagnosis if criteria strongly met, to mitigate harm. **Addiction:** **Change Grow Live** runs “Change Grow Live Gloucestershire” for community treatment, with some input from the trust. **Dual Diagnosis:** Glos has a well-integrated Dual Diagnosis Steering Group (including reps from police, housing, etc.). They acknowledge many clients have *undiagnosed* neurodiversity. A recent local briefing echoed national findings that treating ADHD early could reduce later addiction rates ⁸. This awareness could make the trust and council open to an OTOS pilot linking their adult ADHD team with CGL’s recovery hubs – perhaps offering combined workshops on managing ADHD symptoms in recovery. **Contact:** 0300 421 8100.

South East Region

- **South London & Maudsley NHS FT (SLaM) – ADHD:** Home to the **Maudsley Adult ADHD Clinic**, one of the first and leading services in the UK. SLaM serves Southwark, Lambeth, Lewisham, Croydon – and also takes **national referrals** for complex ADHD cases. Despite its expertise, even SLaM has significant waits (though shorter than many places – children in SLaM’s CAMHS wait ~5 weeks vs 5 years in Belfast) ³⁶. SLaM has been part of research trials for new ADHD treatments and is leading the new NHS England ADHD **Taskforce** (launched 2024 to improve ADHD care ³⁷). **Addiction:** SLaM runs extensive addiction services: the **National Addictions Centre**, inpatient detox, and local community teams (though some borough services were transferred to charities in recent cuts). They operate specialist services for *club drugs*, cannabis psychosis, alcohol assertive outreach, etc. **Dual Diagnosis:** SLaM arguably has the most developed dual diagnosis approach – they coined the term “tri-morbidity” for patients with mental illness, substance use, and physical health issues. However, even here, ADHD+addiction can slip through; historically dual diagnosis efforts centered on schizophrenia or bipolar + heroin/alcohol. With SLaM’s thought-leader status and focus on evidence, an OTOS partnership could be framed as a **research pilot** – e.g. studying outcomes of a coaching intervention for co-occurring ADHD and substance misuse. If successful, SLaM’s endorsement could promote scaling across the NHS. **Contact:** 020 3228 6000.
- **South West London & St George’s Mental Health NHS Trust (SWLSTG) – ADHD:** Serves Kingston, Merton, Richmond, Sutton, Wandsworth. Provides adult ADHD services (St George’s clinic), but like others, demand far outstrips supply. They collaborate with charity ADHD Richmond & Kingston for patient support while waiting. **Addiction:** The SW London boroughs use a mix of providers: e.g. WDP and CGL in different boroughs. The trust itself focuses on mental health inpatient and community care. **Dual Diagnosis:** SWLSTG runs a **24/7 crisis line** for mental health that also anyone misusing substances can call ³⁸. They have “dual diagnosis link workers” in each borough team. There’s room for improvement: e.g. ensuring ADHD assessments are considered when patients repeatedly drop out of addiction treatment (possibly due to inattentiveness or executive function issues). OTOS could help identify such patterns and intervene early. **Contact:** Switchboard **0203 513 5000** ³⁹.
- **Oxleas NHS FT – ADHD:** Covers Greenwich, Bexley, Bromley in SE London. Oxleas offers an adult ADHD service but with long waits (2+ years). They’ve partnered with primary care to do shared care after diagnosis, but getting to diagnosis is slow. **Addiction:** These boroughs’ services are run

by **CGL** (Greenwich), **CASA** (Bexley) and Change Grow Live (Bromley). Oxleas provides some staff for a joint “Aftercare” program supporting recovery. *Dual Diagnosis*: Oxleas has a strong prison mental health team (at HMP Belmarsh, etc.) where many inmates have dual diagnosis – including undiagnosed ADHD (prison studies suggest **25%+ of inmates have ADHD**). They have begun screening in forensic settings. OTOS could potentially work with Oxleas in a criminal justice context: providing support to offenders with ADHD histories as they transition to community, to break the cycle of addiction and re-offending. **Contact**: 01322 625700.

- **Surrey & Borders Partnership NHS FT (SABP)** – *ADHD*: Runs adult ADHD services in Surrey; demand is extremely high especially in areas like Guildford and Woking. Many waited ~2 years; Surrey has recently commissioned additional providers (including an online psychiatry service) to help. *Addiction*: Surrey’s service “**i-access**” is a partnership between SABP and the charity Catalyst. This is noteworthy – an NHS trust jointly running community addiction treatment with a charity. It means NHS staff (including psychiatrists) are directly involved in routine addiction care. *Dual Diagnosis*: SABP’s joint service allows fairly seamless dual diagnosis management: a patient can receive therapy from Catalyst and medication from an SABP psychiatrist under one umbrella. This is a promising setup for integrating ADHD care – indeed, if ADHD is identified, SABP can prescribe stimulant medication while Catalyst provides coaching in daily routines. **Opportunity**: OTOS could augment Surrey’s i-access by providing specialized coaching groups for clients with ADHD, leveraging the existing integrated structure to demonstrate cost-effective benefits (e.g. fewer missed appointments, improved abstinence). **Contact**: 0300 222 5932.
- **Sussex Partnership NHS FT** – *ADHD*: Provides services in East and West Sussex. They have faced criticism for long waits; some patients wait 3–4 years. Sussex has also had issues with *fragmented pathways* – for instance, different eligibility criteria in East vs West Sussex. The trust is now standardizing and trying to bring waits down. *Addiction*: Sussex’s services are delivered by **Change Grow Live** (East Sussex STAR service) and **CGL** (West Sussex). The trust runs some recovery-focused mental health programs that liaise with those teams. *Dual Diagnosis*: Sussex Partnership has a “Dual Diagnosis Lead” and a robust policy, partly spurred by incidents in the past. They emphasize training mental health staff in basic substance misuse interventions. Still, with resources stretched, embedding ADHD expertise hasn’t happened yet. OTOS could collaborate with Sussex on a pilot in, say, Brighton – a city with both a high prevalence of adult ADHD (reflected in vibrant ADHD support communities) and significant substance misuse issues in its population. A targeted program there could yield visible results (e.g. improved engagement at the Brighton Recovery Centre for clients who had ADHD coaching). **Contact**: 0300 304 0100.
- **Kent & Medway NHS & Social Care Partnership Trust (KMPT)** – *ADHD*: Historically, adult ADHD services in Kent were minimal, but in recent years referrals exploded. The NHS commissioned NELFT (as mentioned) to handle ADHD assessments in Kent. Nonetheless, many in Kent still face **5+ year waits**, especially after a pause during COVID. Some have turned to private providers. *Addiction*: Kent’s services are split among providers: CGL, Forward Trust, and We Are With You, different in each district. KMPT focuses on secondary mental healthcare. *Dual Diagnosis*: KMPT has dual diagnosis nurses and works with the Kent Drug and Alcohol Team (a multi-agency). They produced a 2020 strategy citing the need to consider *neurodiversity in dual diagnosis*. However, implementation is nascent. **Opportunity**: Given the patchwork in Kent, OTOS could serve as a unifying element – offering a consistent support service across the county that both NHS and all the various addiction providers could tap into for their service users with ADHD. This could reduce duplication and ensure continuity as people move or transition between services. **Contact**: 01622 724100.

- **Southern Health NHS FT** (Hampshire) – *ADHD*: Provides services in Hampshire (outside of Portsmouth/Southampton). Southern Health has relatively better wait times in some areas (under 1 year in parts of Hampshire) but worse in others. They also faced a scandal in 2022 when some ADHD assessments were found to be done by under-qualified staff – they’ve since corrected this. *Addiction*: Hampshire’s community services are run by **Inclusion (MPFT)** as “Hampshire Recovery Service”. Southern Health works closely with them and runs inpatient detox at Antelope House. *Dual Diagnosis*: Southern Health integrates physical healthcare (they have community hospitals), so their perspective on dual diagnosis also includes physical health aspects (like liver disease management alongside mental health). ADHD isn’t overtly part of this, but as an NHS England pilot site for the new “**community mental health framework**,” they have flexibility to innovate with voluntary partners. OTOS could align with that framework in Hampshire, building on existing community assets (peer support groups, etc.) to support those with co-occurring needs. **Contact**: 0300 123 3390.
- **Solent NHS Trust** (Portsmouth/Southampton) – *ADHD*: In these cities, Solent Trust provides CAMHS and some adult services, but adult ADHD is often referred to Sussex Partnership (for Brighton) or Southern Health (for Hants) due to historical boundaries. As a result, many Portsmouth/Southampton adults have had trouble accessing ADHD diagnosis. This is now improving with local clinics established. *Addiction*: Portsmouth’s service is **Society of St James & Inclusion**; Southampton’s is **Change Grow Live**. Solent Trust does physical health and community nursing, including some alcohol care teams in hospitals. *Dual Diagnosis*: The lack of directly provided mental health by Solent (it’s a community/physical health trust) means formal dual diagnosis programs are handled by external partnerships. OTOS could play a role in bridging physical and mental health here – e.g. supporting patients seen in Solent’s Homeless Healthcare team who often have undiagnosed ADHD and addiction. **Contact**: 0300 123 3392.

London Region

(Many London trusts have been covered above in their regional contexts. Below are a few additional London-specific notes.)

London has a high concentration of services and also high need. **More than 83% of adults with ADHD in London report receiving medication**, the highest in England (vs only ~55% in regions like the Midlands) ⁴⁰, indicating better access to specialist care. Yet, waiting lists remain and many bounce between boroughs if they move. **Dual diagnosis** in London often intersects with homelessness, HIV care (for chemsex users), and the criminal justice system. Public Health England noted London had a lower proportion (25%) of clinicians reporting 2+ year ADHD waits compared to elsewhere (e.g. East of Eng 55%) ⁴¹, but still significant.

Key London trusts and notes:

- **West London NHS Trust** – serves Hounslow, Ealing, Hammersmith & Fulham. It provides ADHD services (with long waits) and runs St Bernard’s Hospital which has a specialist Addiction Psychiatry ward. Could benefit from community ADHD recovery support.
- **Camden & Islington NHS FT (C&I)** – strong adult ADHD team (who collaborate with University College London on research), but capacity issues cause long waits. C&I works closely with **Better Lives (WDP)** addiction service in Islington. A joint initiative to embed ADHD-informed practices there could be fruitful.

- **Barnet, Enfield & Haringey Mental Health Trust (BEH)** – ADHD services available but stretched. Addiction services by Barnet (CGL), Enfield (Barnet, Enfield & Haringey Recovery Service) – partnership opportunities exist with BEH’s community teams.
- **Tavistock & Portman NHS FT** – a specialist trust focusing on therapy. Runs some adult ASD/ADHD psychotherapy groups. They could contribute expertise in psychosocial approaches for ADHD, complementing OTOS coaching in addiction contexts.

In summary, **every region faces challenges in addressing the unique combination of ADHD and addiction**, though specifics vary. Many trusts rely on external providers for addiction treatment, funded via local authorities’ public health budgets rather than NHS – for example, Barnet’s drug service is commissioned by the council to CGL ⁴², and Tower Hamlets’ service (“Reset”) is council-funded with East London NHS FT as a partner ⁴³. This fragmentation often leaves a gap in accountability for dual diagnosis: mental health trusts might say the patient’s substance use must be “stabilized” first, while addiction services may lack resources or remit to diagnose/treat ADHD.

OTOS has the opportunity to position itself as the *solution to bridge this gap* – working across organizational boundaries to support patients through assessment, treatment, and recovery. By partnering with trusts and their commissioned providers, OTOS can help implement the forthcoming recommendations of the NHS England ADHD Taskforce (2024) which is seeking a “joined-up approach” to rising ADHD demand ³⁷. This could include ensuring that those with co-occurring addiction are not left behind.

Contact Details for NHS Mental Health Trusts (England)

The table below lists England’s mental health trusts by region, with their main contact details (headquarters phone and website). These are the primary organizations responsible for NHS ADHD and dual diagnosis services in each area. (Note: Addiction services are often delivered by separate partners, but the trusts below coordinate mental health care and can be initial points of contact for partnership inquiries.)

NHS Mental Health Trust	Region (Area Covered)	Contact (Phone & Website)
Northumberland, Tyne & Wear NHS FT (CNTW)	North East (North East England)	0191 246 6800 ¹¹ – www.cntw.nhs.uk
Tees, Esk & Wear Valleys NHS FT (TEWV)	North East (Teesside, Durham, York)	0191 333 6161 – www.tewv.nhs.uk
Humber Teaching NHS FT	Yorkshire (Hull, E. Riding)	01482 301700 – www.humber.nhs.uk
Leeds & York Partnership NHS FT (LYPFT)	Yorkshire (Leeds, York)	(0113) 305 5000 ¹⁷ – www.leedsandyorkpft.nhs.uk
Bradford District Care NHS FT	Yorkshire (Bradford)	01274 494100 – www.bdct.nhs.uk
South West Yorks Partnership NHS FT (SWYPFT)	Yorkshire (W. Yorks, Barnsley)	(01924) 327000 ¹⁹ – www.southwestyorkshire.nhs.uk

NHS Mental Health Trust	Region (Area Covered)	Contact (Phone & Website)
Sheffield Health & Social Care NHS FT	Yorkshire (Sheffield)	0114 271 6310 – www.shsc.nhs.uk
Rotherham, Doncaster & S. Humber NHS FT (RDaSH)	Yorkshire/Humber (Roth/Doncaster)	03000 213 000 ¹⁶ – www.rdash.nhs.uk
Cumbria, Northumb. Tyne & Wear NHS FT (CNTW)	North East (Newcastle, Cumbria)	0191 246 6800 ¹¹ – www.cntw.nhs.uk (<i>see above</i>)
Greater Manchester Mental Health NHS FT	North West (Greater Manchester)	0161 773 9121 – www.gmmh.nhs.uk
Mersey Care NHS FT	North West (Merseyside, Liverpool)	(0151) 473 0303 ²² – www.merseycare.nhs.uk
Cheshire & Wirral Partnership NHS FT (CWP)	North West (Cheshire, Wirral)	01244 397000 – www.cwp.nhs.uk
Lancashire & South Cumbria NHS FT (LSCFT)	North West (Lancashire)	01772 695300 – www.lscft.nhs.uk
Pennine Care NHS FT	North West (E. Greater Manc.)	0161 716 3000 – www.penninecare.nhs.uk
Birmingham & Solihull Mental Health FT	Midlands (West Mids – B’ham)	0121 301 0000 ²⁴ – www.bsmhft.nhs.uk
Black Country Healthcare NHS FT	Midlands (W. Mids – Black Country)	0345 146 1800 – www.blackcountryhealthcare.nhs.uk
Coventry & Warwickshire Partnership Trust	Midlands (Coventry, Warwickshire)	024 7636 2100 – www.covwarkpt.nhs.uk
Herefordshire & Worcs Health & Care Trust	Midlands (Hereford, Worcs)	01905 760000 – www.hacw.nhs.uk
Derbyshire Healthcare NHS FT	Midlands (Derbyshire)	01332 623700 – www.derbyshirehealthcareft.nhs.uk
Nottinghamshire Healthcare NHS FT	Midlands (Notts)	0115 969 1300 – www.nottinghamshirehealthcare.nhs.uk
Leicestershire Partnership NHS Trust	Midlands (Leicestershire)	0116 295 0030 – www.leicspart.nhs.uk
Lincolnshire Partnership NHS FT	Midlands (Lincolnshire)	01522 309200 – www.lpft.nhs.uk
North Staffs Combined Healthcare NHS Trust	Midlands (Stoke-on-Trent)	01782 275100 – www.combined.nhs.uk
Midlands Partnership University NHS FT (MPFT)	Midlands (Staffs, Shropshire)	0300 790 7000 – www.mpft.nhs.uk

NHS Mental Health Trust	Region (Area Covered)	Contact (Phone & Website)
Northamptonshire Healthcare NHS FT (NHFT)	Midlands (Northants)	01604 682682 – www.nhft.nhs.uk
Cambridgeshire & Peterborough NHS FT	East of Eng (Cams, Peterboro)	01223 219400 – www.cpft.nhs.uk
Norfolk & Suffolk NHS FT	East of Eng (Norfolk, Suffolk)	01603 421421 – www.nsft.nhs.uk
Essex Partnership Univ. NHS FT (EPUT)	East of Eng (Essex)	(01245) 362000 ²⁸ – www.eput.nhs.uk
Herts Partnership Univ. NHS FT (HPFT)	East of Eng (Herts, W. Essex)	01707 253800 – www.hpft.nhs.uk
North East London NHS FT (NELFT)	London/East (NE London, Essex)	0300 555 1200 ³⁰ – www.nelft.nhs.uk
Central & North West London NHS FT (CNWL)	London/SE/East (NW Lon, MK, etc)	(020) 3214 5700 ³³ – www.cnwl.nhs.uk
Avon & Wiltshire Mental Health Partnership	South West (Bristol, Wilts)	01225 325680 – www.awp.nhs.uk
Cornwall Partnership NHS FT	South West (Cornwall, IoS)	01208 834600 – www.cornwallft.nhs.uk
Devon Partnership NHS Trust	South West (Devon)	01392 676676 – www.dpt.nhs.uk
Dorset HealthCare Univ. NHS FT	South West (Dorset)	01202 277000 – www.dorsethealthcare.nhs.uk
Somerset NHS FT	South West (Somerset)	01823 368350 – www.somersetft.nhs.uk
Gloucestershire Health & Care NHS FT	South West (Gloucs)	0300 421 8100 – www.ghc.nhs.uk
South London & Maudsley NHS FT (SLaM)	London (S. London boroughs)	020 3228 6000 – www.slam.nhs.uk
South West London & St George's MH Trust	London (SW London)	020 3513 5000 ³⁹ – www.swlstg.nhs.uk
West London NHS Trust	London (W. London)	020 8354 8354 – www.westlondon.nhs.uk
Camden & Islington NHS FT	London (Camden, Islington)	020 3317 3500 – www.candi.nhs.uk
Barnet, Enfield & Haringey MH Trust	London (N. London)	020 8702 3000 – www.beh-mht.nhs.uk
Oxleas NHS FT	London (Greenwich, Bromley)	01322 625700 – www.oxleas.nhs.uk

NHS Mental Health Trust	Region (Area Covered)	Contact (Phone & Website)
Tavistock & Portman NHS FT	London (Specialist Services)	020 7435 7111 – www.tavistockandportman.nhs.uk
Surrey & Borders Partnership NHS FT	South East (Surrey)	0300 222 5932 – www.sabp.nhs.uk
Sussex Partnership NHS FT	South East (Sussex)	0300 304 0100 – www.sussexpartnership.nhs.uk
Kent & Medway NHS & SC Partnership Trust	South East (Kent)	01622 724100 – www.kmpt.nhs.uk
Southern Health NHS FT	South East (Hampshire)	0300 123 3390 – www.southernhealth.nhs.uk
Solent NHS Trust	South East (Portsmouth/Southampton)	0300 123 3392 – www.solent.nhs.uk

(Note: FT = Foundation Trust. Regions are approximate; some trusts span multiple regions. Contact info is for main headquarters/general enquiries.)

ADHD & Addiction: Regional Figures and Waiting Lists

The table below summarizes available statistics on ADHD and addiction for each NHS England region. It includes estimated **prevalence of ADHD**, current **waiting list sizes**, number of people in **addiction treatment**, and prevalence of **dual diagnosis** (both general and specifically ADHD+addiction). These figures illustrate the scale of need and can help identify where OTOS might have the greatest impact. (Data sources include ADHD UK's 2023 FOI analysis and 2025 updates, NHS Digital/PHE reports, and research findings. Some values are estimates, as indicated.)

Region	Population	Estimated People with ADHD (diagnosed & on treatment)	Waiting for ADHD Assessment (approx.)	Adults in Addiction Treatment (2022–23)	% in Treatment with Mental Health Needs	Estimated with ADHD + Addiction (co-occurring)
North East & Yorkshire	~8 million	~45,000 ^{7 17} (ADHD medication recipients)	~20,000 (FOI extrapolation) – Longest waits: 55% >2yrs ⁴⁶	~44,000 ⁴⁴	~70% ⁴⁵ (≈31,000 with MH needs)	~11,000 (25% of in-treatment) ⁴⁷
North West	~7.3 million	~39,000 (on ADHD meds)	~17,000 waiting – Significant waits (many >2yrs)	~38,000 ⁴⁴	~71% (≈27,000 with MH needs)	~9,500 (est. ADHD+SUD cases)

Region	Population	Estimated People with ADHD (diagnosed & on treatment)	Waiting for ADHD Assessment (approx.)	Adults in Addiction Treatment (2022-23)	% in Treatment with Mental Health Needs	Estimated with ADHD + Addiction (co-occurring)
Midlands (East & West)	~10 million	~60,000 (on ADHD meds)	~26,000 waiting – <i>Worst case: 10+ year waits in parts</i>	~58,000	~67-77% (≈40,000 with MH needs)	~14,500 with ADHD+addiction
East of England	~6 million	~30,000 (on ADHD meds)	~13,000 waiting – <i>Shortest waits: 14% >2yrs (best nationally)</i>	~29,000	~70% (≈20,000 with MH needs)	~7,000 with ADHD+addiction
London	~9 million	~45,000 (on ADHD meds; high diagnosis rate)	~20,000 waiting – <i>Many external assessments used; 25% >2yrs</i>	~44,000	~68% (≈30,000 with MH needs)	~11,000 with ADHD+addiction
South East (incl. Hants)	~9 million	~48,000 (on ADHD meds)	~21,000 waiting – <i>Growing demand; variable waits</i>	~47,000	~69% (≈32,000 with MH needs)	~12,000 with ADHD+addiction
South West	~5.5 million	~30,000 (on ADHD meds)	~13,000 waiting – <i>Moderate waits (Dorset ~12wks best)</i>	~29,000	~72% (≈21,000 with MH needs)	~7,000 with ADHD+addiction
England Total	56 million	~300,000 diagnosed on ADHD meds (2023/24)	~131,000 (est. 2023) to 668,000 (2025 ADHD UK est.) waiting	290,635 in treatment	71% with mental health needs	25% of those in treatment (est. ~72,000)

Key insights from the data: Across England, an **estimated 2.6 million people have ADHD** (all ages), but only a fraction are diagnosed or receiving treatment (around 300k by 2024). ADHD diagnostic demand has exploded – by one estimate, **over 668,000 people were waiting for an**

assessment as of 2025 ⁴⁸. This represents a massive increase from ~131k estimated in 2023 (reflecting new referrals outpacing capacity). NHS waiting lists have grown so long that **some backlogs would take millennia to clear at current rates** (e.g. a trust with only 3 assessments a year and 6,000 waiting = 2,000 year wait mathematically) ¹⁵. This “*constitutional failure*” is prompting legal challenges to force parity for ADHD under the 18-week NHS guarantee ⁵⁰ ⁵¹.

Meanwhile, in 2022–23, **290,635 adults were in treatment for substance misuse in England** ⁴⁴ – a number rising after years of static funding. Roughly **70%** of those entering treatment have a co-occurring mental health need ⁴⁵, yet only **21%** were receiving mental health treatment for it ⁵² (often just via GP). This gap shows how many are not getting integrated care. Importantly, new research confirms that **about 25% of people in addiction treatment meet ADHD criteria** ⁴⁷. In other words, potentially **over 70,000 individuals** in current treatment could have ADHD+addiction – a population that often doesn’t fully engage with standard services due to attentional and executive function difficulties.

Regionally, there is variation: Northern England reports higher rates of unmet need (e.g. **55% in North-East/Yorks waiting 2+ years for ADHD care** ⁴⁶), whereas some southern regions had slightly better access (London and East of England had the lowest proportions waiting >2 years) ⁴⁶. However, even in “better” areas, absolute numbers waiting are large and dual diagnosis needs are significant.

Conclusion & Opportunities for OTOS: The alignment of these data points to a pressing need for innovative solutions. Every trust and region is grappling with how to serve the growing cohort of individuals with co-occurring ADHD and addiction. Many trusts acknowledge the uniqueness of this dual diagnosis – for example, clinicians note that what looks like “resistance” in addiction treatment may in fact be untreated ADHD symptoms ⁵. Yet, due to siloed services and resource constraints, few have implemented targeted programs. OTOS can position itself as a partner to fill that void:

- **By aiding trusts in reducing waits** – perhaps offering interim assessments or psychoeducation for those on ADHD waiting lists who are also at risk of substance misuse, preventing “lost years” where patients self-medicate.
- **By training and embedding recovery coaches** who understand ADHD in existing addiction services (mostly charity-run) – improving engagement and completion rates. Given that **patients with untreated ADHD often struggle to adhere to rigid program structures** ⁵, a coach can personalize strategies to keep them on track.
- **By collaborating on new care pathways** – for instance, creating joint protocols where an ADHD diagnosis triggers a fast-track into appropriate substance misuse support, or vice versa, ensuring truly “no wrong door” in accessing care.

Crucially, OTOS should emphasize outcomes like reduced relapse, improved functioning, and patient satisfaction, using the language of both NHS mental health and public health commissioners. With national bodies like NHS England and the Dept. of Health now paying attention (via the ADHD Taskforce and new drug strategy funding), the timing is ideal for OTOS to demonstrate a model that can be scaled up.

In summary, the landscape analysis shows **significant gaps but also readiness**: trusts are aware that ADHD+addiction is not the “average” dual diagnosis and requires specialized approaches. By providing that specialization in a flexible, partnership-oriented way, OTOS could become an invaluable ally to NHS trusts and their external providers, ultimately improving care for a cohort that has been under-served for too long.

Sources:

- ADHD prevalence, risks, and co-morbidity: Rehabs UK report (2023) ¹ ; News Letter (Belfast) article ⁴⁷ ⁴ .
 - ADHD waiting times and regional disparities: ADHD UK FOI Report (2023) ⁹ ³⁶ ; Pharmaceutical Journal (Apr 2024) ⁴¹ ; Neurodiversity Directory (Oct 2025) ¹⁵ ⁴⁸ .
 - Benefit of ADHD treatment (meds) on outcomes (incl. substance abuse): Neurodiversity Directory ⁷ .
 - Percentage of addiction treatment clients with mental health needs and receiving care: OHID Adult Substance Misuse Stats (2023) ²⁹ ⁵² .
 - Proportion of addiction clients with ADHD: News Letter ² (reflecting Addictive Behaviors study 2020).
 - Specific trust data (Sheffield screening 97% out; Dorset 12-week wait best; Belfast worst child wait; Hereford/Worcs 10-year wait): ADHD UK report ¹⁴ ⁹ ³⁶ .
 - NHS commissioning of addiction services (e.g. Barnet, Tower Hamlets examples): NHS England / FRANK ⁴² ⁴³ .
 - Contact details for trusts: NHS and CQC official listings ²⁴ ¹⁶ ³⁰ ²² ¹⁹ ¹⁷ .
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<https://gps.northcentrallondon.icb.nhs.uk/services/change-grow-live>
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